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RESPONSE

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Response to Vogelstein: How the 2012 AAP Task Force on circumcision went wrong

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Abstract

Vogelstein cautions medical organizations against jumping into the fray of controversial issues, yet proffers the 2012 American Academy of Pediatrics' Task Force policy position on infant male circumcision as 'an appropriate use of position-statements.' Only a scratch below the surface of this policy statement uncovers the Task Force's failure to consider Vogelstein's many caveats. The Task Force supported the cultural practice by putting undeserved emphasis on questionable scientific data, while ignoring or underplaying the importance of valid contrary scientific data. Without any effort to quantitatively assess the risk/benefit balance, the Task Force concluded the benefits of circumcision outweighed the risks, while acknowledging that the incidence of risks was unknown. This Task Force differed from other Academy policy-forming panels by ignoring the Academy's standard quality measures and by not appointing members with extensive research experience, extensive publications, or recognized expertise directly related to this topic. Despite nearly 100 publications available at the time addressing the substantial ethical issues associated with infant male circumcision, the Task Force chose to ignore the ethical controversy. They merely stated, with minimal justification, the opinion of one of the Task Force members that the practice of infant male circumcision is morally permissible. The release of the report has fostered an explosion of academic discussion on the ethics of infant male circumcision with a number of national medical organizations now decrying the practice as a human rights violation.

KEYWORDS

circumcision, cultural bias, policy statement

I read with interest the recent article by Vogelstein cautioning medical organizations against jumping into the fray of controversial issues.[1] I was struck that the author would hold out the American Academy of Pediatrics (AAP) 2012 position on male circumcision[2] as 'an appropriate use of position-statements,' because when one looks carefully at the report, a very different conclusion should be reached.

The author is correct in stating 'This position is based largely on scientific data,' but the author may not be aware of the now well-documented selective gathering and selective use of scientific data employed by the Task Force. To reach their conclusion that 'the benefits of newborn male circumcision outweigh the risks,' the Task Force gave undeserved emphasis to scientific data that supported this conclusion while ignoring a large swath of scientific data that do not support this conclusion and ignored actual, substantial ethical controversies. For example, the Task Force refused to consider any case reports or case-series in their analysis. Since the most serious complications associated with circumcision (including deaths) are primarily reported as case reports or in case-series, these risks would be underestimated both subjectively and objectively. Next, studies with results that presented circumcision in a less than favorable light were either excluded or, in many cases, erroneously judged to be of lower quality. For example, among studies of similar designs the Task Force systematically assigned higher quality ratings to studies that were favorable to circumcision than to those that were not favorable.[3] In fact, the Task Force chose not to use

[1]Vogelstein, E. (2016). Professional hubris and its consequences: Why organizations of health-care professions should not adopt ethically controversial positions. *Bioethics*, *30*(4), 234–243.

[2]Task Force on Circumcision. (2012). Male circumcision. *Pediatrics*, *130*(3), e756–e785.

[3]One example, out of many, is the Task Force's assessment of the quality of studies assessing the frequency of adverse outcomes. Based on study design, studies that prospectively collect data are considered to be more reliable than retrospective chart reviews, which in turn are considered to be more reliable than studies based on information extracted from databases. The

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the standard quality assessment method that the AAP had adopted in 2004,[4] but instead used a less standard quality assessment procedure. In this manner the Task Force was able to formulate a conclusion that was consistent with the prevailing cultural norm in the United States, but which is inconsistent with that reached by all other peer medical associations in other countries (see below). Unfortunately, such bias and lack of scientific rigor may have put the AAP at risk for a lawsuit.[5]

While the AAP 'plausibly has the scientific and clinical *bona fides* to pronounce upon the issue' and has a strong track record in making pronouncements on other issues, the work of the 2012 Task Force falls well below the AAP's typical high standard.

When the AAP released policy statements on sinus infections, ear infections, and urinary tract infections, the membership of the committees included experts with vast research experience, extensive publications, and expertise directly related to these topics.[6] The publication history on the topic of circumcision for the eight members of 2012 Circumcision Task Force included one original research study published in 1987,[7] and three opinion pieces,[8] one of which may have been solicited because of the author's membership on the Task Force.[9] One of the Task Force members, Dr. Andrew Freedman, stated in an interview that he had circumcised his own son on his parents' kitchen table, 'not for medical reasons . . . but because I had 3,000 years of ancestors looking over my shoulder.'[10] Another of the Task Force members was the Chair of the Committee on Bioethics that concluded that some forms of ritualized female genital cutting were ethically permissible without any discussion of a child's right to bodily integrity, even though all forms of nontherapeutic female genital cutting are banned by federal law.[11] In the firestorm that ensued, the AAP 'retired' this position a month later.[12] With a steadfast proponent of parental authority, rather than children's interests, as the Committee on Bioethics's representative to the Task Force,[13] it was unlikely that any serious, let alone nuanced, discussion of the child's right to bodily integrity would take place.

Instead of acknowledging, confronting, and discussing the ongoing controversy surrounding the moral permissibility of infant male circumcision,[14] the Task Force blithely acted as though there was no controversy by simply failing to mention opposing views. This approach back-fired, however, as the discussion about whether or not infant male circumcision is morally permissible or even a human rights violation has exploded since the AAP reports with several national medical organizations characterising it as an impermissible violation of children's rights.[15]

Such one-sidedness does not promote the constructive debate Vogelstein believes should be aimed for. As the author notes 'greater and more rigorous reflection and dialogue is more likely to result in the truth.' The evidence suggests, however, that the Task Force was not interested in the 'truth,' but rather in appealing to their own authority to use this policy-statement as, what the author calls, an 'argumentative bludgeon' to leverage financial reimbursement from Medicaid and other third-party payers for performing the procedure. Indeed, the repeated solicitation for funding for circumcision throughout the report comes across as a non-sequitur.

prospective study cited by the Task Force documented a relatively high incidence of adhesions, balanitis, and meatal stenosis following circumcision and was assessed to be of 'poor' quality by the Task Force. The cited chart review view study, undertaken by the Centers for Disease Control and Prevention, reported a 3.2% rate of adverse outcomes and was assigned a 'fair' rating. The cited study based on database information reported a 0.2% rate of adverse outcomes and was awarded a 'good' rating. These relative ratings are the exact opposite of what would be expected based on study design but exactly what would be expected if one had a bias toward demonstrating that infant male circumcision is a low risk procedure. Christakis, D. A., et al. (2000). A trade-off analysis of routine newborn circumcision. *Pediatrics*, *105* (1), 246–249; O'Brien, T. R., Calle, E. E., & Poole, W. K. (1995). Incidence of neonatal circumcision in Atlanta, 1985–1986. *Southern Medical Journal*. *88*(4), 411–415; Van Howe RS. (1997). Variability in penile appearance and penile findings: A prospective study. *British Journal of Urology*, *80*(5), 776–782.

[4]American academy of pediatrics steering committee on quality improvement and management. (2004). Classifying recommendations for clinical practice guidelines. *Pediatrics*, *114*(3), 874–877.

[5]Giannetti, M. R. (2000). Circumcision and the American Academy of Pediatrics: Should scientific misconduct result in trade association liability. *Iowa Law Review*, *85*(4), 1507–1568.

[6]Wald, E. R., et al. (2013). Clinical practice guideline for the diagnosis and management of acute bacterial sinusitis in children aged 1 to 18 years. *Pediatrics*, *132*(1), e262–e280; Lieberthal, A. S., et al. (2013). The diagnosis and management of acute otitis media. *Pediatrics*, *131*(3), e964–e999; Subcommittee on Urinary Tract Infection, Steering Committee on Quality Improvement and Management. (2011). Urinary tract infection: Clinical practice guidelines for the diagnosis and management of the initial UTI in febrile infants and children 2 to 24 months. *Pediatrics*, *128*(3), 595–609.

[7]Maxwell, L. G., et al. (1987). Penile nerve block for newborn circumcision. *Obstetrics & Gynecology*, *70*(3 Part 1), 415–419.

[8]Maxwell, L. G., & Yaster, M. (1999). Analgesia for neonatal circumcision: No more studies, Just Do It. *Archives of Pediatrics & Adolescent Medicine*, *153*(5), 444–445; Diekema, D. S. (2009). Boldt v. Boldt: A pediatric ethics perspective. *Journal of Clinical Ethics*, *153*(3), 251–257; Brady, M. T. (2010). Newborn circumcision: Routine or not routine, that is the question. *Archives of Pediatrics & Adolescent Medicine*, *164*(1), 94–96. Erratum (2010) in: *Archives of Pediatrics & Adolescent Medicine*, *164*, 397.

[9]Brady, *op. cit*. note 8.

[10]Merwin, T. (2012, September 19). Fleshing out change on circumcision. *New York Jewish Week*, http://jewishweek.timesofisrael.com/features/new-york-minute/fleshing-out-change-circumcision [Accessed December 29, 2016]

[11]Committee on Bioethics. (2010). Policy statement — ritual genital cutting of female minors. *Pediatrics*, *125*(5), 1088–1093.

[12]American Academy of Pediatrics Withdraws Policy Statement on Female Genital Cutting [press release]. (2010, May 27). Elk Grove Village, IL: American Academy of Pediatrics. Available at: http://www.aap.org/advocacy/releases/fgc-may27-2010.htm [Accessed May 31, 2010]; MacReady, N. (2010). AAP retracts statement on controversial procedure. *Lancet*, *376*(9734), 15.

[13]Gunther, D. F., & Diekema, D. S. (2006). Attenuating growth in children with profound developmental disability: A new approach to an old dilemma. *Archives of Pediatrics & Adolescent Medicine*, *160*(10), 1013–1017.

[14]Nearly 100 publications on the ethical/legal permissibility of infant male circumcision were available for the Task Force to consider prior to the release of their report. Most notable were a heated debate in the *American Journal of Bioethics* in 2003 and an entire issue of the *Journal of Medical Ethics* in 2004. For a list of publications see the Addendum provided as online appendix.

[15]Most notable are issues being devoted to the topic in 2013 by *Global Discourse* and *the Journal of Medical Ethics* and in the *Journal of Law, Medicine & Ethics* in 2016. An interesting debate also took place in the *American Journal of Bioethics* in 2015. For a list of publications see the Addendum provided as online appendix. The positions of national medical organization are discussed below.

If the members of the Task Force were 'experts in ways directly relevant to determining the risks and benefits of procedures like circumcision, in particular through clinical experience and by evaluating and putting into context relevant scientific evidence,' then why does the AAP stand alone in its conclusion? National medical and pediatric organizations in Canada, Australasia, the Netherlands, Germany, Slovenia, Sweden, Norway, Denmark, Finland, and South Africa, along with the Nordic Ombudsmen, and the Council of Europe all had access to the same body of knowledge as the members of the AAP Task Force, yet did not reach the same conclusion.[16]

It is striking that Vogelstein would mention only in passing that the AAP policy statement 'has its detractors,' without citing any of the relevant criticisms or contrary policies.[17] With the AAP standing so far afield from the rest of the world,[18] one wonders whether the members of the 2012 Task Force were privy to information or insights that the majority of their peers and counterparts in other nations are missing?

Most unsettling is that their conclusion, 'that the health benefits of newborn male circumcision outweigh the risks' does not stand on its own logical feet. If the benefits outweigh the risks, then the Task Force should have recommended the procedure, but the Task Force does not recommend the procedure. Not surprisingly, circumcision proponents have used the Task Force's pronouncement that the benefits outweigh the risks as a tool to promote circumcision, often failing to mention, as the author does, that the Task Force failed to recommend the procedure.

Furthermore, the report does not directly compare the benefits and risks because it was impossible for them to do so. Several times in the report the Task Force acknowledges that '[t]he true incidence of complications after newborn circumcision is unknown.' One can take this a step further, as Darby has, by observing that the Task Force only tallied the 'complications' following the procedure without considering the intrinsic harms/losses associated with losing a heathy and functional body part.[19] This lack of knowledge could result in a failure to recognize the procedure's intrinsic harms/losses. If the risks *and* harms of the procedure have not been quantified, how did the Task Force determine that they were outweighed by the benefits? Moreover, how and on what basis did the Task Force assign values or weights to individual risks and benefits, so that they could be meaningfully balanced against one another? Frisch and Earp note that this cannot be done by the fiat of a committee:

> consider the example of a modest reduction in the absolute risk of contracting certain sexually transmitted infections, a health benefit that is frequently attributed to – primarily adult – circumcision [. . .] what is the weight or value that one should assign to this particular benefit? For example, should it be considered 'worth' the risk [. . .] of a surgical mishap [. . .] the loss of the penile prepuce [. . .] [or] removing too much penile skin [. . .] leading to painful erections later in life? [. . .] The answer to these questions cannot be 'objectively' determined. Instead,

[16]Sorokan, S. T., Finlay, J. C., Jefferies, A. L.; Canadian Paediatric Society Fetus and Newborn Committee, Infectious Diseases and Immunization Committee. (2015). Newborn male circumcision. *Paediatrics & Child Health*, *20*(6), 311–315; Royal Australasian College of Physicians, Paediatrics & Child Health Division. (2010). Circumcision of infant males. https://www.racp.edu.au/docs/default-source/advocacy-library/circumcision-of-infant-males.pdf [Accessed December 29, 2016]; Royal Dutch Medical Association. (2010). *Non-therapeutic Circumcision of Male Minors*. KNMG; Hartman, W. (26 Nov 2012) 'Stellungnahme Dr.med. Wolfram Hartmann, Präsident des Berufsverbands der Kinder- und Jugendärzte, zur Anhörung am 26. November 2012 zum Gesetzentwurf der Bundesregierung: 'Entwurf eines Gesetzes über den Umfang der Personensorge bei einer Beschneidung des männlichen Kindes' und zum Gesetzentwurf der Abgeordneten Marlene Rupprecht, Katja Dörner, Diana Golze, Caren Marks, Rolf Schwanitz, weiterer Abgeordneter: 'Entwurf eines Gesetzes über den Umfang der Personensorge und die Rechte des männlichen Kindes bei einer Beschneidung''. www.kinderaerzte-im-netz.de/bvkj/kinpopup/psfile/pdf/70/121126_Ste50aa5e211e6a6.pdf [Accessed 7 April 2013]; Slovenian Human Rights Ombudsman. (2010, February). Circumcision of boys for non-medical reasons is a Violation of Children's Rights. www.varuh-rs.si/medijsko-sredisce/aktualni-primeri/novice/detajl/obrezovanje-fantkov-iz-nemedicinskih-razlogov-je-krsitev-otrokovih-pravic [Accessed November 1, 2012]; Guiborg C. (2012, February 19). Swedish docs in circumcision protest. *The Local*. http://m.thelocal.se/39200/20120219/ [Accessed November 7, 2012]; Lægeforeningen. (2016, December). Lægeforeningens politik vedrørende omskæring af drengebørn unden medicinsk indikation. http://www.laeger.dk/nyhed/download/docs/F29714/LF%20politik%20vedr%C3%B8rende%20drengeomsk%C3%A6ring_1.%20december___ENDELIG.pdf; Suomen Lääkäriliitto. (2012). Poikien ympärileikkaus. http://www.laakariliitto.fi/uutiset/ kannanotot/ymparileikkaus.html [Accessed November 7, 2012]; Letter to Dean Ferris from Ulundi Behrtel dated 23 June 2011 from South African Medical Association Human Rights, Law & Ethics Committee; Nordic Ombudman News Release. (2013, September 30). MALE CIRCUMCISION: Nordic Ombudsman Will Seek a Ban on Non-therapeutic Male Circumcision. www.crin.org/resources/infodetail.asp?id=31830 Accessed October 28, 2013; Council of Europe. (2013, October 1). Children's Right to Physical Integrity. http://www.assembly.coe.int/nw/xml/XRef/Xref-DocDetails-EN.asp?FileID=20176&lang=EN

[17]Svoboda, J. S., Adler, P. W., & Van Howe, R. S. (2016). Circumcision is unethical and unlawful. *Journal of Law, Medicine & Ethics*, *44*, 263–282; Svoboda, J. S., & Van Howe, R. S. (2013). Out of step: Fatal flaws in the latest AAP policy report on neonatal circumcision. *Journal of Medical Ethics*, *39*(7), 434–441; Frisch, M., et al. (2013). Cultural bias in the AAP's technical report and policy statement on male circumcision. *Pediatrics*, *131*(4), 796–800; Darby, R. (2015). Risks, benefits, complications and harms: Neglected factors in the current debate on non-therapeutic circumcision. *Kennedy Institute of Ethics Journal*, *25*(1), 1–34; Frisch, M, & Earp, B. D. (2016). Circumcision of male infants and children as a public health measure in developed countries: A critical assessment of recent evidence. *Global Public Health*. epub ahead of print. http://www.tandfonline.com/doi/full/10.1080/17441692.2016.1184292

[18]Clinical ethicist Melanie Jansen characterizes the AAP position as an 'extreme position' and 'an outlier in its interpretation of the evidence for and against routine circumcision.' She notes that, 'One possible explanation for the significantly different recommendations of the AAP compared with multiple other similar bodies is the profit-driven context in which many of its members practise, and the lucrative nature of 'routine' minor surgical procedures done in privately funded system.' Jansen, M. (2017). Still locked: A reply to wodak, Ziegler and Morris. *Journal of Paediatrics and Child Health*, *53*(1), 93–94.

[19]Darby, *op. cit*. note 17. This omission may be attributed to ignorance. After the Task Force released its report, two of the Task Force members, at a symposium in South Carolina, revealed their lack of knowledge of even some very basic circumcision related topics, such as the anatomy and functions of the male foreskin. Svoboda et al, *op. cit*. note 17. 2016.

> they will depend upon such factors as how much value one places on having intact versus modified genitalia, how willing one is to engage in safe sex practices (even if one is circumcised), and how much risk one feels comfortable taking on when it comes to a surgery performed on a physically and symbolically sensitive part of one's body.[20]

Indeed, in a later editorial, Task Force member Andrew Freedman conceded the point: in addition to having 'insufficient information about the actual incidence and burden of nonacute complications,' he wrote, the AAP's 2012 assessment of benefits versus risks was also hampered by the 'lack of a universally accepted metric to accurately measure or balance the risks and benefits.'[21]

Even the metrics that have been devised were not used. The Task Force showed no evidence of performing a cost analysis, a cost-benefit analysis, a cost-effectiveness analysis, or a cost-utility analysis. Instead, they reached their conclusion without providing any direct comparison of the benefits and risks whatsoever. When confronted with this shortcoming,[22] the AAP backpedaled by stating, 'These benefits were *felt* to outweigh the risks of the procedure.'[23]

Is it 'an appropriate use of position-statements' when a policy is based on the subjective feelings of an 8-member committee rather than scientific evidence in the apparent service of cultural or other non-scientific objectives? As Freedman stated in his editorial:

> Most circumcisions are done due to religious and cultural tradition. In the West, although parents may use the conflicting medical literature to buttress their own beliefs and desires, for the most part parents choose what they want for a wide variety of nonmedical reasons. There can be no doubt that religion, culture, aesthetic preference, familial identity, and personal experience all factor into their decision [. . .] Given the role of the phallus in our culture, it is not illegitimate to consider these realms of a person's life in making this nontherapeutic, only partially medical decision. . . . The ethical standard [we] used was 'the best interest of the child,' and in this setting the well-informed parent was felt to be the best proxy to pass this judgment. *Protecting this option was not an idle concern at a time when there are serious efforts in both the United States and Europe to ban the procedure outright*.[24]

Thus, Freedman makes clear that the AAP Task Force was not engaged in a dispassionate exercise in assessing and balancing medical benefits and risks, but was set on reaching a conclusion that was aimed at blatantly non-scientific ends . . . precisely the sort of thing that Vogelstein is objecting to in his article. In conclusion, the stance taken by the AAP on male circumcision was not a success but a failure. It did not learn its lesson from the failed female genital cutting policy of 2010. It did not give guidance to North American physicians who, like the majority of their European colleagues, believe infant male circumcision is a human rights violation. The AAP would have been much better off issuing a policy on the care of intact genitals in males and to begin developing a strategy for how distance itself from a cultural practice that is widely regarded as harmful outside of the communities that are invested in it, and is rejected by majority of the world. Instead, they took a reactionary approach of cultural relativism and circled their wagons.

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SUPPORTING INFORMATION

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[20]Frisch & Earp, *op. cit*. note 17.

[21]Freedman, A. L. (2016). The circumcision debate: Beyond benefits and risks. *Pediatrics*, *137*, epub ahead of print.

[22]Svoboda & Van Howe, *op. cit*. note 17.

[23]The AAP Task Force on Circumcision 2012. (2013). The AAP task force on neonatal circumcision: A call for respectful dialogue. *Journal of Medical Ethics*. *39*(7), 442–443. (Emphasis mine).

[24]Freeman, *op. cit*. note 21.